



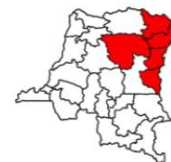
EBOLA PRESIDENTIAL FORCE TASK 17

Ministry of Public Health, Hygiene and Social Welfare

National Institute of Public Health

Public Health Emergency Operations Center

MVE17 Incident Management System



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°102/MVEBDB/08/24/2026

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HIGHLIGHTS (last 24 hours)

As of August 24, 2026, there were 72 new confirmed cases of Ebola Virus Disease, including 18 community deaths. (EVM) were recorded in the provinces of Ituri (61 cases), North Kivu (7 cases), Haut Uélé (3) and Bas-Uélé (1). A new health zone (Ganga) in Bas-Uélé has been affected in the last 24 hours.

ACCUMULATIVE CASE 5,656	ACCUMULATIVE DEATH CONFIRMED 2,715	LETHALITY 48.0%	PATIENTS IN ISOLATION/CTE 792	ACCUMULATIVE HEALED 1,245	TRACKING RATES CONTACTS (Of the Day) 80.9%
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Affected Provinces and Affected Health Zones



6 Provinces Affected

Haut-Uélé, Ituri, North Kivu, South Kivu, Tshopo & Bas Uélé



58 out of 151 health zones affected (38.4%)



Summary of key points from the last 24 hours

In the past 24 hours, **72 new confirmed cases**, including **18 community deaths** from Ebola Virus Disease (EVD), have been reported in the provinces of **Ituri (61 cases)**, **North Kivu (7)**, **Haut Uélé (3)** and **Bas Uele (1)**.

Over the past 24 hours, **30 patients** have been declared recovered after receiving two negative Ebola virus (EV) test results (23 in Ituri, 4 in North Kivu, and 3 in Haut-Uélé). On the other hand, **17 deaths within Ebola treatment centers** were recorded, including 16 in Ituri and 1 in North Kivu. in Haut Uélé.

From the start of the epidemic until August 24, 2026, **5,656 confirmed cases, including 2,715 deaths**, were reported in the DRC, representing a case fatality rate of **48.0 %**. Ituri remains the epicenter of the epidemic, accounting for 83.4% of cumulative confirmed cases and 84.7% of new cases reported in the last 24 hours.

A new health zone (Ganga, in Bas-Uélé) has been affected in the last 24 hours. As a result, 58 of the 151 health zones in the six provinces are now affected since the start of the epidemic in the DRC. Ituri has 28 out of 36 affected health zones, followed by North Kivu (13/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34), and Bas-Uélé (3/11).

Distribution of confirmed cases and deaths by affected province

(As of August 24, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4716	2,119	44.9% 28/36	(77.8%) 13/34	61
North Kivu	735	501	68.2%	(38.2%) 6/13	7
Haut-Uélé	184	85	46.2%	(46.1%) 7/23	3
Tshopo	15	8	53.3%	(30.4%) 1/34	0
South Kivu	3	1	33.3%	(2.9%) 3/11	0
Bas Uélé	3	1	33.3%	(27.3%)	1
Total	5,656	2,715	48.0%	58/151 (38.4%)	72

Confirmed cases and deaths by province and health zone (situation as of August 24, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4716	2119	44.9%	61	15	16	31
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	8	2	25.0%				0
Aungba	12	5	41.7%				0
Bamboo	139	29	20.9%	12	1		1
Boga	2	2	100.0%	1	1		1
Bunia	1312	411	31.3%	14	6		6
Damascus	27	9	33.3%				0
Drodoro	13	6	46.2%				0
Fataki	73	30	41.1%				0
Gethy	5	2	40.0%				0
Kambala	2	0	0.0%				0
Kilo	32	12	37.5%				0
Komanda	61	39	63.9%	2	1		1
Lita	204	111	54.4%	7	2		2
Logo	11	5	45.5%				0
Lolwa	18	4	22.2%				0
Mahagi	4	1	25.0%				0
Mambasa	14	7	50.0%				0
Mandima	32	19	59.4%				0
Mangala	203	109	53.7%	6	3		3
Mongbwalu	603	284	47.1%	1	1		1
Nia-Nia	193	108	56.0%				0
Nizi	622	252	40.5%	10	0		0

Nyankunde	123	35	28.5%	1	0		0
Rimba	10	4	40.0%				0
Rwampara	920	335	36.4%	7	0		0
Tchomia	55	25	45.5%				0
To ventilate*	N / A	266	N / A	N / A	N / A	16	16
North Kivu	735	501	68.2%	7	3	0	3
Blessed	118	84	71.2%	1	0	0	0
Butembo	140	107	76.4%	2	1	0	1
Goma	1	0	0.0%				0
Kalunguta	15	7	46.7%				0
Katwa	351	236	67.2%	3	2	0	2
Kyondo	16	12	75.0%				0
Lubero	1	1	100.0%				0
Mabalako	3	2	66.7%				0
Masereka	7	4	57.1%				0
Musienene	70	41	58.6%				0
Mutwanga	1	1	100.0%				0
Oicha	7	4	57.1%	1	0	0	0
Vuhovi	5	2	40.0%				0
Haut-Uélé	184	85	46.2%	3	0	1	1
Boma Mangbetu	26	13	50.0%				0
Gombari	3	1	33.3%				0
Isiro	70	30	42.9%	2	0	1	1
Pawa	20	15	75.0%				0
Rungu	1	0	0.0%				0
Wamba	64	26	40.6%	1	0		0
Tshopo	15	8	53.3%	0	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	3	1	33.3%				0
Lubunga	1	0	0.0%				0
Makiso-Kisangani	6	4	66.7%				0
Mangobo	2	2	100.0%				0
Tshopo	1	0	0.0%				0
Wanie-Rukula	1	1	100.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%				0
Bas Uélé	3	1	33.3%	1	0	0	0
Buta	1	1	100.0%				0
Viadana	1	0	0.0%				0
Ganga	1	0	0.0%	1	0	0	0
Total	5656	2715	48.0%	72	18	17	35

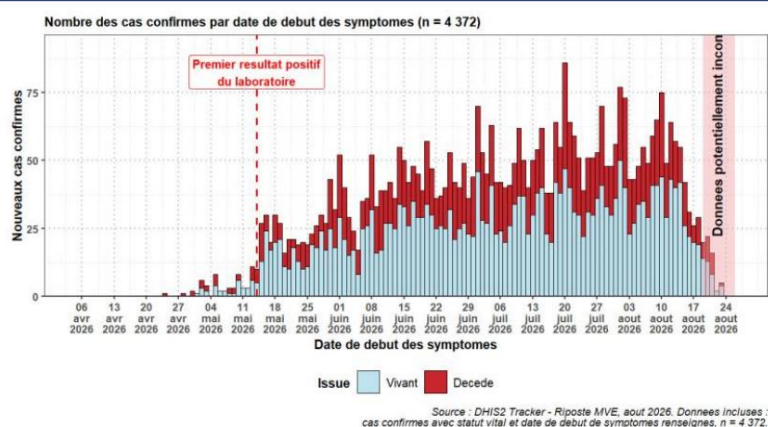
*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

NA: Not Applicable.

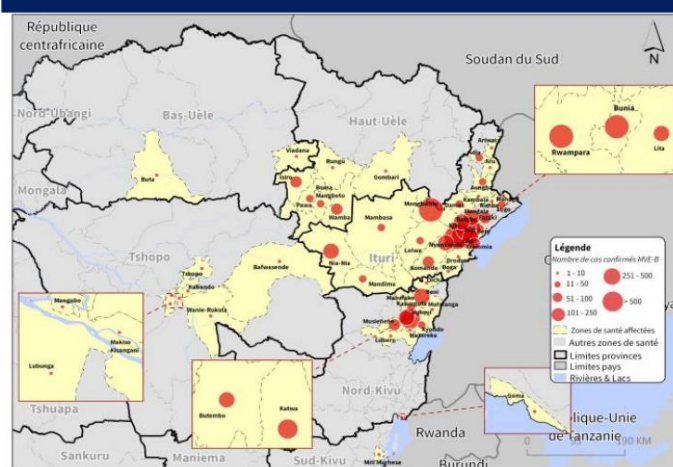
Status of alerts notified by province, as of August 24, 2026

DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (no suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Haut-Uélé	20	2	22	20	2	0	0	22	20
Bas Uélé	18	0	18	3	0	15	0	3	2
Ituri	614	50	664	103	38	423	4	141	87
North Kivu	946	66	1012	79	58	836	0	137	63
South Kivu	4	0	4	1	0	3	0	1	0
Tshopo	80	0	80	7	0	71	0	7	3
Total General	1682	118	1,800	213	98	1348	4	311	175

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : Supervise the work of developing zonal correction plans and training 50 local supervisors of the community response in the nine affected health zones.
- **Ituri** : Continued supervision of response activities and holding of the meeting of the coordination.
- **Tshopo** : Coordination meeting held and supervision of activities continued of the response.
- **South Kivu** : Continued intensification of surveillance and active search in the Ibanda and Nyangezi health zones; the province has gone 74 days without a new confirmed case and the last one dates back to June 10, 2026.

- **Bas-Uélé:** supervise the investigations around the new confirmed case in the Agri-Uélé health area (Ganga Health Zone).

1.1.2. Epidemiological surveillance

1.1.3. Main Actions

By the end of the day on August 24, 2026, 1,800 alerts had been recorded, including 1,663 (92.4%) were verified in the six affected provinces. Following these verifications, 311 (18.7%) were validated as suspected cases and all were investigated, including 175 were transferred to the CT/CTE.

The proportion of contacts followed up in the last 24 hours is 80.9% (19 495 views out of 24,106 to follow), below the 85% threshold: Ituri 87.2% (11,865/13,611), Tshopo 100.0% (489/489) and North Kivu 73.3% (5,903/8,053). Haut-Uélé 64.3% (1,105/1,719) and Bas-Uélé 56.8% (133/234) show low follow-up rates.

1.1.4. Challenges

Weak follow-up of contacts around confirmed cases mainly in the provinces of Haut Uélé and Bas Uélé linked to a lack of teams dedicated to these activities and insufficient completeness in Ituri (11 health zones did not transmit data on contact follow-up).

1.2. Update on checkpoint and point of entry (PoC/PoE) activities

1.2.1. Main actions

- Twelve (12) alerts were notified to PoE/PoC in the last 24 hours: five in Ituri, including four live alerts validated and transferred to CT/CTE (100%) and one lifeless body intercepted at the Mangina PoC (ZS Komanda) not swabé for security reasons; three in North Kivu, one validated at the Foner Kasindi PoC, and one in Bas-Uélé (PK2 Titule PoC), validated and investigated within two hours and Three alerts were also notified to Tshopo.

Indicators	Ituri	North Kivu	South Kivu	Tshopo	Haut-Uélé	Bas-Uélé	Overall
Report completeness	98.3% (59/60)	100% (18/18)	55.5% (5/9)	85% (17/20)	54.5% (6/11)	22.2% (4/18)	80.1% (109/136)
% of strategic PoE/PoC functional	9.09% (1/11)	N / A	N / A	N / A	N / A	ND	9.1% (1/11)
Number of people who have switched to PoE/PoC	150,290	90,879	10,657	2,925	9080	793	264,624
% of travelers screened	98.0%	97.77% 98.4%	99.5% 97.0%			100%	97.9%
% of travelers who washed the hands	97.2%	97.77% 98.4%	97.7% 97.0%			100%	97.5%
% of travelers made aware	99.5%	97.77%	100% 100%	97.0% 78.8%			98.8%
Number of alerts notified	5	3	0	3	0	1	12
Number of live alerts validated (suspected cases)	4	1	0	3	0	1	9
Number of bodies recovered today	1	0	0	0	0	0	1
% of suspected cases transferred to CT/CTE	100% (4/4)	100% (1/1)	0% 66.6%	(2/3)	0%	0% (0/1)	77.8% (7/9)
% of lifeless bodies swabed/secured	0% (0/1)	0%	0%	0%	0%	0%	0% (0/1)

% of suspected cases investigated within 24 hours	100% (5/5)	100% (3/3)	0%	100% (3/3)	0%	100% (1/1)	100% (12/12)
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1.2.2. Challenges

Insufficient operationalization of PoE/PoC: national completeness only reaches 80.1% (109 reports out of 136), with low levels in Bas-Uélé (22.2%) and Haut-Uélé (54.5%).

In Ituri, 1 (9.1%) of the 11 strategic PoE/PoCs is operating without interruption; in Haut-Uélé, five PoCs (Dusu, Apodo, Aungba, PK6 Niangara and Gossamu) did not operate due to a strike by unpaid service providers; Tshopo and South Kivu have partial complete reporting, with 17/20 and 5/9 PoE/PoCs respectively having transmitted their data.

1.3. Laboratory

1.3.1. Main actions

- **Ituri : 61 new positive results** (46 living and 15 deaths) out of 347 new samples received and analyzed (**positivity: 17.6 %**).
- **North Kivu : 7 new positive results** (4 living and 3 dead) out of 99 samples analyzed (**positivity of 7.1 %**).
- **Haut-Uélé: 3 new positive results** (all live) out of 18 samples analyzed (**positivity of 16.7%**).
- **Bas-Uélé : 1 new positive result** out of 4 samples analyzed (**positivity 25%**).

1.4. Infection Prevention and Control (IPC/EDS)

1.4.1. Main actions

- **In Ituri**, 23 rings were opened out of the 59 identified (39.0%), 76 households decontaminated out of 98 identified (77.5%) and 7 out of 7 FOSA (100%); 5 FOSA assessments were carried out.
- **In North Kivu**, 6 rings were opened in Beni, 17 health facilities and 5 households were decontaminated; 200 health facilities were monitored and supported, and 689 service providers were briefed. The EDS pillar received 44 alerts (15 reports): 44 bodies were successfully swabbed, and 44 EDS were carried out.
- **In Bas-Uélé**, 11 of the 13 targeted health establishments (HEs) received support, and 11 HEs remain under infection prevention and control (IPC) surveillance; 3 infection rings were opened (Buta, Viadana, and Ganga), and the Ganga General Referral Hospital (HGR) was assessed (card score of 32.8%). No death alerts were received, and no disease surveillance was conducted.
- **In Tshopo**, 16 community health workers (CHWs) were supported in 3 health zones (Makiso-Kisangani, Mangobo, and Kabondo), where 138 healthcare providers were briefed; the functionality of 15 handwashing stations and 16 triage units was monitored. Training began for 35 hygiene workers (5 teams) from 4 health zones, and a health data collection (HDC) was conducted at the Makiso General Referral Hospital morgue.

1.4.2. Challenges

- Weak implementation of PCI standards: in Ituri, ring opening remains limited to 39.0% and 2 swab failures were recorded; one member of the EDS team died in a traffic accident (ZS Bambu) and two others were attacked in Rwampara (AS Makabo).
- In Bas-Uélé, the decontamination of the household of relatives of the first confirmed case in Buta has still not been possible due to persistent community resistance, the

pre-breakdown of several PCI inputs delaying the briefing of the 500 service providers in the ZS of Aketi, Likati, Bondo, Monga and Bili.

- In North Kivu, resistance to dignified and safe burials persists in Katwa, Butembo, Kalunguta and Beni, and the road to the Bundji cemetery (ZS Beni) remains impassable.

1.5. Continuity of care

1.5.1. Main actions

- In **Ituri**, 76 new admissions were recorded and 109 discharges, including 23 recoveries. Thus, 522 patients are hospitalized in 978 beds, representing an overall occupancy rate of 53.4%.
- In **North Kivu**, 32 admissions were recorded, with 35 discharges, including 4 recoveries. At the end of the day, 178 patients were hospitalized for 206 beds, representing an overall occupancy rate of 86.4%.
- In **Tshopo**, there were 4 new admissions of suspected cases and 10 patients isolated out of 14 beds, representing 71.4% occupancy.
- In **Bas Uélé**, 1 confirmed case is in isolation at HGR Ganga.
- In **Haut Uélé**, 12 admissions were recorded, with 3 discharges, including 3 recoveries. A total of 81 patients are in isolation out of 130 beds, or 62.3%.

1.5.2. Challenges

Low reception capacity and absence of CTEs in some health zones; saturation of the CTEs in Bambu (confirmed), Kilo and Kigonze (suspected) in Ituri and exceeding the capacity in suspected beds in North Kivu (143 patients for 142 beds).

1.6. Risk communication and community engagement

1.6.1. Main actions

- In **Ituri**, 246 contacts were identified around confirmed cases (85 in Rwampara, 124 in Lolwa, and 37 in Kilo); 9 EDS (Educational and Health Diagnosis) sessions were facilitated, 13 families were made aware of the risks, and 7 positive results were announced to families. In total, 10,303 people were reached by communication activities (5,645 through home visits, 3,175 through educational talks, 1,423 through mass awareness campaigns, and 60 through advocacy and community dialogue); 22 radio programs and 6 radio spots were produced, 36 community radio stations were involved, and 42 IEC (Information, Education, and Communication) materials were distributed.
- In **Tshopo**, 1,272 people were sensitized during home visits in 212 households in Makiso, Mangobo and Kabondo; spots were broadcast on 6 radio and television channels, managers and agents of the Provincial Directorate of Education were sensitized and a rumor was clarified.
- In **North Kivu**, 8 cases were covered and 42 household heads affected in 8 health zones; 17 rumors were clarified, 300 youth leaders sensitized in Butembo and 3 recovered from the Katwa CTE testified before the community.
- In **Haut-Uélé**, 2,853 people were reached by communication activities: 1,517 through home visits to 312 households, 1,279 through educational talks, and 57 through mass awareness campaigns. Five local radio stations broadcast the messages, and 53 IEC materials were distributed, bringing the total to 381.

- **In Bas-Uélé**, an awareness session on Ebola virus disease, prevention measures and dignified and safe burial was organized for the youth group on Mobenge Avenue in Buta, along with the distribution of a handwashing device.

1.6.2. Challenges

Persistence of community challenges: in Ituri, refusals to sample (Nia-Nia), to swab (Alimasi) and to list contacts (Kilo State, 48 contacts) were recorded; only 8 of the 28 affected health zones in Ituri and 2 in North Kivu reported community concerns, i.e. 19% of the affected areas in the country (compared to 23% in the previous period).

In Haut-Uélé, the response teams were attacked in Isiro and Pawa.

1.7. Mental Health and Psychosocial Support (MHPSS)

1.7.1. Main actions

- **In Ituri**, 24 new confirmed cases and 108 confirmed cases under follow-up benefited from SMSPS interventions, as well as 24 new suspected cases and 120 suspected cases under follow-up; 76 households received support, 47 people living with HIV were assisted, 39 children were monitored in the community, and 55 test results were announced, including 24 positive results. Twenty out of 23 recovered patients (87%) have been reintegrated into the community.
- **In North Kivu**, all new confirmed cases and 65 of the 94 new suspected cases (65.6%) benefited from SMSPS interventions; 87 results announced out of 134 expected (64.9%) were made, 93 PPLs and 72 affected households were supported and 13 separated or orphaned children were followed up in the community.
- **In Haut-Uélé**, one new confirmed case and 27 confirmed cases under observation received psychosocial support, as well as one new suspected case, 32 caregivers, and 29 people with disabilities; 11 test results were announced, including 3 positive results. Coverage remains limited to 2 of the 6 affected health zones (33%), due to a lack of sufficient psychosocial support workers.
- **In Tshopo**, 2 new suspected cases and 2 suspected cases under follow-up received an interview and psychological support, as well as 17 caregivers of hospitalized cases; 3 laboratory results were announced, none of which were positive, and 406 people were reached by psychoeducation in religious settings.

1.7. Logistics

1.7.1. Main actions

- In North Kivu, 13 tonnes of PCI/WASH inputs delivered by UNICEF to Butembo and Katwa and six 4x4 vehicles for the health zones of Butembo, Katwa and Beni; continuation of construction work on the Matanda CT.

1.8. Security

1.8.1. Main actions

- **In North Kivu**, the pillar provided support to two PCI teams (CS Dokotolo/AS Butsili and CS Ngalika/AS Ruwenzori) and support to EDS teams at the Bundji cemetery (100% completion); the permanent security of all morgues in the city of Beni was decided after the Ngalika morgue was stoned and a body was removed by the population.

- ##### 1.8.2. In Ituri
- the securing of strategic PoCs remains incomplete: a lifeless body was intercepted
The PoC in Mangina (ZS Komanda) could not be swabé due to security reasons.

1.8.3. Challenges

In Ituri, animal health teams are unable to collect samples due to a lack of training, while 29 dead animals have been investigated and four new cases of bovine hematuria reported in Berunda. In South Kivu and Haut-Uélé, the demotivation of stakeholders due to non-payment persists and has led to the suspension of five Points of Control (PoCs) in Haut-Uélé.

1.9. PSEA (Prevention of Sexual Exploitation and Abuse)

1.9.1. Main actions

Continued support for training of community, military and police leaders on PSEA and MVE, jointly with the communications pillar.

1.9.2. Challenges

- Still insufficient coverage of PSEA activities, limited to 4 out of 36 health zones in Ituri, lack of mapping of stakeholders, insufficient mass awareness materials and lack of transport and computer equipment for the pillar; Coverage remains limited to 4 of the 36 health zones of Ituri (11.1%) and only two partners (WHO and Africa CDC) report their PSEA activities.

KEY MESSAGES

- Ebola virus transmission remains active, with 72 new cases including 18 community deaths and 17 intra-CTE recorded in the last 24 hours, bringing the national total to 5,656 confirmed cases and 2,715 deaths.
- Ituri remains the epicenter of the epidemic, concentrating 83.4% of cumulative confirmed cases and 84.7% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance shows good performance (100.0%) of suspected cases investigated while the follow-up rate (80.9%) is below the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of funds.



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